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Office Visit - Sep 02, 2026

with Tatiana Portela at UT Southwestern Primary Care at Frisco



Notes from Care Team



Progress Notes by Tatiana Portela at 9/2/2026 10:00 AM

HISTORY

CHIEF COMPLAINT: Medical Question/problem (Pt is here to f/u from ER. Pt was in a MVA and wants to know how to navigate her pain.)

VISIT TYPE: Acute Visit

History of Present Illness

A 48-year-old female with chronic PTSD, and chronic spinal stenosis seen for neck pain and back/hip pain after a motor vehicle collision.

Motor vehicle collision occurred on 08/28/2026. Another vehicle struck her car on the side, and she reports head strike. Visible injuries were not present initially, but bruising developed the next day around the neck and knee. Neck pain involves the left side and limits range of motion. Back pain is worst near the spine and adjacent bones, extends into the hip, and worsens with pressure.

Pain is described as beyond her baseline chronic pain from CRPS and chronic stenosis. Current ketorolac, tramadol, and methocarbamol cause drowsiness without meaningful relief. Fentanyl in the hospital relieved pain for less than 1 hour before it returned. Pain is making driving difficult, and she is not currently able to drive.

She reports concern about increased intracranial pressure after the accident. This was initially thought to be headache-related, but she states it is not a headache. She reports pineal gland calcification since childhood and states prior CT scans had not shown this current finding.

Leg muscle spasms occur at night and wake her from sleep. She uses potassium supplements and electrolyte drinks because she reports becoming potassium deficient quickly and has had low potassium for as long as she can remember.

Social History:

Occupation: Driver

CT HEAD WO CON, CTA HEAD NECK ANGIO COMBO

REPORT DATE: 8/30/2026

ORDERING PHYSICIAN: SHANNON KELSEY DELLA MONICA

HISTORY: mvc, severe headache.

COMPARISON(S): CT head without contrast 10/15/2025; CTA head/neck 8/17/2023. TECHNIQUE: CT of the head without contrast. CT angiogram of the head and neck after administration of intravenous iodinated contrast. This examination was performed according to

ALARA principles. The estimated degree of maximum luminal diameter stenosis is according to the NASCET standard. 3D and/or MIP reformatted images were provided per CTA protocol.

FINDINGS:

CT HEAD:

Brain: Brain parenchymal attenuation and morphology are normal. No acute hemorrhage. Gray-white matter differentiation is maintained. No mass effect or midline shift.

Ventricles/CSF spaces: Unchanged partially empty sella. Otherwise the ventricles and extra-axial CSF spaces appear normal.

CTA NECK:

Aorta: There is no visualized aortic aneurysm or dissection. There is a four-vessel origin of the aortic arch with the left vertebral artery arising directly from the aorta. The great vessel origins are normal.

Right Carotid: No occlusion, stenosis, or dissection of the common carotid and cervical internal carotid arteries.

Left Carotid: No occlusion, stenosis, or dissection of the common carotid and cervical internal carotid arteries.

Vertebral Arteries: The origins of the vertebral arteries are patent. There is no vertebral artery occlusion, dissection, or significant stenosis

CTA HEAD

Anterior Circulation: Normal in course and caliber. No high-grade stenosis or occlusion. No aneurysm or vascular malformation.

Posterior Circulation: Normal in course and caliber. No high-grade stenosis or occlusion. No aneurysm or vascular malformation.

OTHER:

Orbits: There is mild cortical tortuosity of the optic nerves with prominent CSF in the optic nerve sheaths. Otherwise normal.

Paranasal sinuses: Imaged paranasal sinuses are clear.

Mastoids/middle ears: Clear.

Calvarium/skull base: No acute osseous abnormality of the calvarium or skull base.

Spine: No acute cervical spine fracture or suspicious osseous lesion. There is mild/moderate appearing multilevel cervical spondylosis without any high-grade osseous narrowing of the cervical spinal canal or neural foramina.

Scalp/facial soft tissues: The paraspinal muscles are unremarkable. The parotid and submandibular glands are normal. The thyroid gland is normal. No cervical lymphadenopathy.

Lung apices: The imaged lung apices are clear.

Impression:

IMPRESSION:

1. No acute intracranial abnormality.
2. No large vessel occlusion, dissection, aneurysm, or significant stenosis involving the major cervical or intracranial arteries.
3. There are multiple findings suggestive of chronic idiopathic intracranial hypertension, which may explain the history of headache.

CT THORACIC SPINE RECONSTRUCTED IMAGES, CT LUMBAR SPINE RECONSTRUCTED IMAGES

HISTORY: MVC, head strike, headache, neck pain, chest pain, left clavicle pain, left knee pain, right knee pain.

TECHNIQUE: Reduced field of view axial, coronal and sagittal image reformats of thoracic and lumbar spine obtained based off of the contrast enhanced CT chest, abdomen and pelvis exam performed today.

All CT scans are performed using dose optimization techniques as appropriate to the exam being performed. These techniques include automatic exposure control and/or standardized protocols utilizing dose matching according to exam type and patient size.

COMPARISON: None

FINDINGS:

Thoracic and lumbar vertebra have normal heights and alignment. No acute fracture or destructive osseous lesion.

Punctate focus sclerosis inferior aspect T10 vertebra may be bone island.

Moderate disc space narrowing and anterior endplate spurring most thoracic levels, mild to moderate disc narrowing and endplate spurring most lumbar levels.

No significant spinal canal stenosis or foraminal stenosis in the thoracic spine.

No significant spinal canal stenosis in the lumbar spine. Mild facet hypertrophy and foraminal bulge cause mild foraminal stenosis on left at L3-L4, at least moderate left greater than right foraminal stenosis at L4-5.

Moderate left facet arthrosis/hypertrophy at L5-S1.

The paraspinal soft tissues are within normal limits.

See report from CT chest, abdomen and pelvis for detailed description of intrathoracic, intra-abdominal and pelvic findings.

IMPRESSION:

1. No acute fracture or subluxation in the thoracic or lumbar spine.
2. Thoracic and lumbar spondylosis with multilevel disc narrowing and endplate spurring.
3. Mild foraminal stenosis on left at L3-L4, at least moderate left greater than right foraminal stenosis at L4-5.
4. Moderate left facet arthrosis/hypertrophy at L5-S1.

Patient Active Problem List

Diagnosis

- DVT (deep vein thrombosis) in pregnancy

- Thrombophilia (*)
- Leukocytosis
- Erythrocytosis

PMH/PSH/Social history/Alcohol and Tobacco Use:

Past Surgical History:

Procedure	Laterality	Date
• APPENDECTOMY		
• COLONOSCOPY		
• COLONOSCOPY	N/A	12/05/2024
<i>Performed by Smith, Ioana Batran, MD at UH 02 Endosc</i>		
• EGD		
• EGD	N/A	12/05/2024
<i>Performed by Smith, Ioana Batran, MD at UH 02 Endosc</i>		
• TUBAL LIGATION		01/23/2014

She reports that she has been smoking cigarettes. She has never used smokeless tobacco. She reports that she does not drink alcohol and does not use drugs.

Medications

Outpatient Encounter Medications as of 9/2/2026

Medication	Sig
• albuterol (PROAIR, PROVENTIL, VENTOLIN HFA) 90 mcg/Actuation aerosol inhaler	Inhale 180 mcg (2 Puffs total) in lungs every 4 hours as needed for Wheezing
• ALPRAZolam	Take 1 Tablet (1 mg total) by mouth

(XANAX) 1 mg oral tablet	three times daily
• dextroamphetamine-amphetamine (ADDERALL) 30 mg oral tablet	Take 30 mg by mouth twice daily
• EPINEPHrine 0.3 mg/0.3 mL intramuscular auto-injector	Inject 0.3 mg in muscle
• gabapentin (NEURONTIN) 300 mg oral capsule	PLEASE SEE ATTACHED FOR DETAILED DIRECTIONS
• ketorolac (TORADOL) 10 mg oral tablet	Take 10 mg by mouth every 6 hours
• methocarbamol (ROBAXIN) 750 mg oral tablet	Take 1 Tablet (750 mg total) by mouth three times daily as needed for Muscle spasms for up to 15 days
• methylPREDNISolone (MEDROL, PAK,) 4 mg dose pack	Use as directed.
• ondansetron (ZOFTRAN ODT) 4 mg rapid dissolve oral tablet	DISSOLVE 1 TABLET (4 MG TOTAL) BY MOUTH TWICE A DAY
• traMADol (ULTRAM) 50 mg oral tablet	Take 50 mg by mouth every 6 hours
• atorvastatin (LIPITOR) 40 mg oral tablet	Take 40 mg by mouth daily
• budesonide (PULMICORT) 0.5	MIX 1 VIAL WITH 2 PACKETS OF SLENDA TO CREATE SLURRY

mg/2 mL suspension	AND SWALLOW TWICE A DAY TO TREAT EOSINOPHILIC ESOPHAGITIS (Patient not taking: Reported on 9/2/2026)
• busPIRone (BUSPAR) 30 mg oral tablet	Take 30 mg by mouth three times daily (Patient not taking: Reported on 9/2/2026)
• clopidogreL (PLAVIX) 75 mg oral tablet	Take 75 mg by mouth daily
• cyanocobalamin (VITAMIN B-12) 1,000 mcg/mL injection	INJECT 1 ML (1,000 MCG TOTAL) IN MUSCLE EVERY MONTH (Patient not taking: Reported on 9/2/2026)
• [DISCONTINUED] methocarbamol (ROBAXIN) 500 mg oral tablet	Take 1,000 mg by mouth
• oxyCODONE, immediate release, (OXY-IR) 10 mg oral tablet	Take 10 mg by mouth twice daily as needed for Pain
• pantoprazole EC (PROTONIX) 40 mg DR oral tablet	TAKE 1 TABLET (40 MG TOTAL) BY MOUTH TWICE DAILY BEFORE BREAKFAST AND DINNER DO NOT CUT, CRUSH, OR CHEW TABLETS. (Patient not taking: Reported on 9/2/2026)
• rifAXIMin (XIFAXAN) 550 mg oral tablet	Take 1 Tablet (550 mg total) by mouth three times daily (Patient not taking: Reported on 9/2/2026)

Allergies

Allergen

- Acetaminophen

Cerner Allergy Text Annotation: Tylenol

Reactions

Anaphylaxis and Swelling

- Doxycycline
throat swelling

Anaphylaxis

- Dust Mites

Respiratory distress

- Oxycodone

Anaphylaxis

W TYLENOL

- Hydrocodone

- Eggshell Membrane

Other Reaction(s): Unknown

- Hydrocodone-Acetaminophen

Other Reaction(s): Unknown

Only allergic to Acetaminophen, not Hydrocodone

Cerner Allergy Text Annotation: Norco

- Shellfish

- Shellfish Containing Products

Other Reaction(s): Unknown

- Aspirin-Cinnamedrine-Caffeine

Hives

Other Reaction(s): Unknown

Only allergic to Tylenol containing midol

also throat swelling

ROS: See HPI

PHYSICAL EXAM

Vitals:

09/02/26 1039

BP: 117/86

BP Site: Upper Arm, Left

Pulse: 97

SpO2: 96%

Weight: 247 lb (112 kg)

Height: 5' 6" (1.676 m)

Physical Exam**Constitutional:**

Appearance: Normal appearance.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Pulses: Normal pulses.

Dorsalis pedis pulses are 2+ on the right side and 2+ on the left side.

Heart sounds: Normal heart sounds. No murmur heard.

Pulmonary:

Effort: Pulmonary effort is normal. No respiratory distress.

Breath sounds: Normal breath sounds. No wheezing.

Musculoskeletal:

General: Normal range of motion.

Right lower leg: No edema.

Left lower leg: No edema.

Skin:

General: Skin is warm and dry.

Neurological:

General: No focal deficit present.

Mental Status: She is alert and oriented to person, place, and time.

Motor: No weakness.

Gait: Gait normal.

Psychiatric:

Mood and Affect: Mood normal.

Behavior: Behavior normal.

Relevant Records Reviewed Today: I have reviewed the EMR for relevant encounter notes, labs, imaging, procedures and specialist notes.

Results

The following results were reviewed by myself.

Imaging

- CT scan of lumbar spine: Mild foraminal stenosis or spinal stenosis.

ASSESSMENT AND PLAN

Melody was seen today for medical question/problem.

Diagnoses and all orders for this visit:

Motor vehicle accident (victim), initial encounter

Neck pain

Spinal stenosis of lumbar region, unspecified whether neurogenic claudication present

Other orders

- methylPREDNISolone (MEDROL, PAK,) 4 mg dose pack; Use as directed.
- methocarbamol (ROBAXIN) 750 mg oral tablet; Take 1 Tablet (750 mg total) by mouth three times daily as needed for Muscle spasms for up to 15 days

Assessment & Plan

1. Motor vehicle accident

Recent MVC on 08/28/2026 with persistent pain. Outside records were reviewed.

- 7-day steroid pack was discussed to help symptoms. Short course and possible blood sugar increase were reviewed.
- Start Medrol Dosepak.
- Increase muscle relaxant dose.
- Continue ketorolac 10 mg every 6 hours as needed.
- Continue after completing medrolpk
- Con tramadol 50 mg every 6 hours as needed for severe pain.
- Continue methocarbamol with higher dose sent to pharmacy.
- Follow up with spine specialist.
- Follow up with neurologist.

2. Cervicalgia

Neck pain limits movement after accident.

- Muscle relaxer benefit for muscular spasms was discussed.

3. Low back pain

Back pain worsened after accident. Prior CT showed mild foraminal or spinal stenosis.

- Follow-up with spine team was discussed.

4. Increased intracranial pressure

Concern raised after head injury. Prior neurology care noted.

- Image review with neurology was discussed.
- Schedule neurology follow-up.

5. Muscle spasm

Nocturnal leg spasms are ongoing.

- Higher muscle relaxant dose was discussed for spasms.

Follow-up: Return within 1 month

No follow-ups on file.

This visit documentation includes usage of ambient artificial intelligence technology. The use of this technology has been explained to the patient, who consented.

35 cumulative minutes spent on day of service providing care for this patient with evaluation and management services. This included pre-visit planning, charting & review of clinical documents, F2F visit encounter, documentation, entering orders, education & counseling, results interpretation and/or post-visit care coordination.

Signed:

Tatiana Portela, ANP

Adult Nurse Practitioner

UTSW Division of Internal Medicine

PRIMARY CARE PHYSICIAN: Uddin, Sarah, MD

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